

Antibiotics for Trauma (if available)

History

- What was the mechanism of injury – blunt (MVC, fall, blow to body vs. penetrating (stabbing, GSW, foreign body)?
- Other medical conditions? Did a medical condition contribute to the mechanism of injury?
- Medications – Taking aspirin, any blood thinners or anticoagulants?

Key Concepts

- Treat life-threatening injuries, pain, perform splinting, etc., before administration of antibiotic that is being given to reduce risk of infection

MARCHES Protocol

- **Massive bleeding control** - Tourniquet/ Wound packing
- **Airway** – NPA/OPA/ Cricothyrotomy
- **Respiratory** – decompress chest if tension pneumothorax / hemothorax suspected, occlusive dressing for open pneumothoraces
- **Circulation**- IV/IO, pelvic binder, reassess tourniquet/wound packing
- **Hypothermia care**/ Hike vs Helicopter (consider evac options)
- **Eye injuries** – cover with rigid shield and no pressure on the eye
- **Spinal motion restriction** if indicated

Criteria

- Adult and pediatric trauma patients with an open fracture of the extremities, extensive soft tissue trauma, life/limb threatening penetrating trauma, or animal/human bite wounds

EMT/AEMT

- Follow Trauma General Patient Care Guideline

Paramedic

Adult Dosing and Administration

- **Ceftriaxone (Rocephin)** 2 grams slow IV/IO push

Contraindications:

- **Ceftriaxone shall not be used in patients with allergy to Cephalosporin Antibiotics**
- History of **Penicillin Allergy IS NOT a contraindication** administration as cross reactions are rare